



STRATEGY & OPERATIONS

Vytalix Group Structure, Ownership, Governance And Mission

Vytalix company-build documentation · reference
02_GROUP_STRUCTURE

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

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Owner: Chief Strategy Officer · **Status:** PROPOSED v1 · **Date:** 7 September 2026 · **Governing file:**

/FACTS_BASE.md (labels KNOWN / VERIFIED / ASSUMPTION / ESTIMATE / PROPOSED / TO VALIDATE apply throughout). Nothing in this document is legal advice; every structural and tax point is TO VALIDATE with a solicitor and an accountant before execution.

Executive view

- 1. Incorporate one company now, not a group.** A single English company — recommended name "Vytalix Group Ltd" subject to clearance — with the four pillars as internal divisions is the only structure that a pre-revenue, solo-founder business can run, fund under SEIS/EIS and explain to investors. A holding-company/subsidiary structure is a Phase 2 decision with explicit triggers (§1.3), not a Day 1 decision.
- 2. MaternaLink IP must be inside that company before anyone is told Vytalix "owns" it.** Of the three routes with David Agunede — IP assignment for equity, licence, or joint venture — assignment for vested equity is the only one investors will accept without a discount; a licence is the fallback; a JV is not recommended (§3). A solicitor must be instructed before any equity is promised.
- 3. Founder holds 100% at incorporation; the first 12 months of dilution is planned, not improvised:** 10–15% option pool, 5–12% for the MaternaLink IP/co-founder settlement (PROPOSED range, vesting), 8–15% for a £150k–£300k SEIS/EIS pre-seed (ESTIMATE). Illustrative cap tables in §2.
- 4. Governance is a credibility asset at this stage.** A two-person statutory board (founder + one independent) and a six-profile advisory board — obstetrics, midwifery/digital midwife, NHS digital/commissioning, Nigerian health-system leadership, regulatory/clinical safety, health-tech scaling — are specified with recruitment routes and compensation ranges (§4). No advisor is currently appointed (KNOWN).
- 5. The name is not yet safe to spend on.** "Vytalix" collides phonetically with Vytalyx Inc. (US health-tech), Vytalize Health (US) and VITALIX LTD (two UK companies). A five-step clearance action with a decision deadline of Day 10 and two reserve names is set out in §7. Vision and five mission variants are finalised in §6 so that copy is ready the day the name clears.

1. Legal structure

1.1 Options considered

Option	Description	Pros	Cons	Fit today
A. Single company, divisions as trading names	One Ltd (E&W). Advisory, Consulting, Health Solutions (MaternaLink), E-Learning are divisions with separate cost centres, not entities.	Cheapest (~£50 incorporation + ~£1.5k–£3k/yr accounts, ESTIMATE); one bank account, one VAT registration, one SEIS/EIS application; investors see one cap table; grants (Innovate UK, SBRI) need a single UK registered applicant.	Services and product liabilities share one balance sheet; a later carve-out of MaternaLink requires a reorganisation (hive-down) with tax and legal cost; a co-founder holding equity "for MaternaLink" in effect holds equity in the services business too.	Recommended for months 0–12

Option	Description	Pros	Cons	Fit today
B. Holding company + operating subsidiaries from Day 1	Vytalix Group Ltd (TopCo) owns 100% of Vytalix Services Ltd and Vytalix Health Solutions Ltd.	Clean ring-fencing of product risk; MaternaLink equity and IP isolated; easier sale/spin-out of either side.	Two to three sets of accounts, intra-group agreements, transfer-pricing, VAT group; SEIS/EIS advance assurance for a holding company depends on subsidiaries being qualifying 90% subsidiaries and the group's trade qualifying (TO VALIDATE with accountant); investors dislike minority holders in subsidiaries; management overhead the founder cannot afford now.	Not now
C. Two unrelated companies (Vytalix services; MaternaLink product with David Agunede as co-owner)	Founder holds services company alone; product company co-owned.	Keeps David Agunede's interest away from the services business; simple for him.	Splits the equity story; Vytalix cannot claim MaternaLink as "its" product; cross-charging of founder time; investors would have to invest in two entities; the "Technology for Life" group narrative dissolves.	Not recommended
D. Nigerian entity now	Register a Nigerian company (CAC) for MaternaLink Nigeria.	Needed eventually for state contracts, NDPA registration, local hiring.	Premature: no contract, no funding, no local team; cost and compliance burden; use a local implementing partner until a signed pilot exists.	Trigger-based (\$1.3)

1.2 Recommendation (PROPOSED)

- **Incorporate "Vytalix Group Ltd"** (reserve: "Vytalix Health Technologies Ltd") as a private company limited by shares in England and Wales, with Model Articles amended for (i) share classes, (ii) drag/tag, (iii) pre-emption, (iv) board reserved matters. SIC codes (TO VALIDATE): 70229 (management consultancy), 62012 (business and domestic software development), 85590 (other education), 86900 (other human health activities).
- **Divisions as trading names**, each with a P&L owner, a cost centre code and a charter (\$5): Vytalix Advisory, Vytalix Consulting, Vytalix Health Solutions, Vytalix E-Learning. The product is referred to as "MaternaLink (in development), a product of Vytalix Health Solutions".
- **All IP is assigned to Vytalix Group Ltd** — founder IP by a founder IP assignment at incorporation (standard, often overlooked), MaternaLink IP by the David Agunede agreement (\$3), contractor IP by clause in every contractor agreement. No IP sits with individuals.
- **Registrations in month 1** (all TO VALIDATE for exact requirements): Companies House; HMRC (Corporation Tax; PAYE when first payroll; VAT registration once turnover approaches the threshold or voluntarily for B2B credibility); ICO data-protection fee (Tier 1, £40–£60/yr ESTIMATE); SEIS/EIS advance assurance; professional indemnity (£1m–£2m) and public liability insurance; a business bank account.

1.3 When and why to separate MaternaLink into a subsidiary

Create **Vytalix Health Solutions Ltd** (or "MaternaLink Ltd") as a 100%-owned subsidiary when **any two** of the following triggers occur, and in any case before the first live deployment holding real patient data:

#	Trigger	Why it forces separation
T1	First live deployment processing identifiable patient data (UK trust, private provider, or African programme)	Ring-fence clinical, data-protection and product-liability exposure away from the services business; separate DSPT/Cyber Essentials/ISO 27001 scope; separate insurance (product liability, cyber).

#	Trigger	Why it forces separation
T2	External equity earmarked for the product only (an investor who wants product exposure without the consultancy)	Investors price a SaaS/impact product and a services business differently; a subsidiary allows product-only investment (but keep TopCo as the preferred investment vehicle wherever possible).
T3	Grant or programme contract that requires a dedicated entity or separate accounts (some donor programmes, some state contracts)	Compliance and audit-trail requirements.
T4	Any regulated status (MHRA registration as a manufacturer for SaMD; medical-device QMS ISO 13485)	The legal manufacturer should be the entity whose whole purpose and QMS is the device.
T5	A product co-founder/CTO whose equity is negotiated on product value only	Cleaner than TopCo equity only if T1–T4 also apply; otherwise use TopCo equity with a product-linked vesting milestone.

Mechanics (TO VALIDATE with solicitor/accountant): hive-down of the MaternaLink IP and contracts from TopCo to the subsidiary by intra-group assignment; intra-group services agreement for founder time; the subsidiary licenses the "Vytalix" brand from TopCo; SEIS/EIS position preserved if the subsidiary is a qualifying 90% subsidiary (TO VALIDATE). Cost ESTIMATE £3k–£6k legal plus ~£2k/yr additional accounting.

Nigeria/Africa entities: create a local subsidiary or joint-venture company only when a signed pilot agreement or funded programme exists; until then contract through a local implementing partner (see [/docs/12_PARTNERSHIP_STRATEGY.md](#)). Typical trigger: first contract > £100k/year or a state requirement for a locally registered vendor (TO VALIDATE per state).

2. Share structure

2.1 Principles (PROPOSED)

1. Founder holds 100% of ordinary shares at incorporation; every later allocation is written, vested and conditional.
2. One share class (ordinary) until the first priced round; SEIS/EIS investors receive ordinary shares (full-risk equity is a condition of relief — TO VALIDATE current rules).
3. Every equity grant to a person who is not investing cash (co-founder, advisor, early employee) vests: 4 years, 1-year cliff, monthly thereafter; good-leaver/bad-leaver provisions; acceleration only on exit (single trigger, 50%; double trigger, 100%) — standard terms, TO VALIDATE with solicitor.
4. Options are granted under an EMI scheme once the company qualifies (fewer than 250 employees, gross assets ≤ £30m, qualifying trade — TO VALIDATE; note that consultancy-heavy trades can be scrutinised).
5. No equity is promised verbally. A signed term sheet precedes any statement to a third party that someone is a "co-founder" or "shareholder".

2.2 Illustrative capitalisation (all figures PROPOSED / ESTIMATE)

Authorised and issued at incorporation: 10,000,000 ordinary shares of £0.0001 nominal (£1,000 total nominal), 100% founder. Share count is arbitrary and chosen for granularity.

Stage	Founder	MaternaLink IP settlement (David Agunede, vesting)	Option pool (EMI, unallocated)	SEIS/EIS pre-seed investors	Total	Notes
Incorporation (month 0)	100%	—	—	—	100%	Founder IP assignment signed.
After IP settlement (month 2–4)	88–92%	8–12% (PROPOSED range; see §3)	—	—	100%	Only if Option 1 (assignment) is chosen; if Option 2 (licence) the equity component is 2–5% plus royalty.
After pool creation (month 4–6)	76–82%	7–11%	12%	—	100%	Pool sized for a CTO/product lead (4–6%), clinical lead (1–2%), advisors (0.25–0.5% each x 5), first 3 hires (2–3%).
After pre-seed (month 9–15): £250k at £2.0m pre-money (ESTIMATE)	68–73%	6–10%	10.7%	11.1%	100%	Pre-money is a placeholder; a pre-revenue UK health-tech at this stage typically prices between £1.5m and £4m (ESTIMATE, TO VALIDATE against comparable rounds). SEIS caps £250k per company lifetime (TO VALIDATE current limit).

Sensitivity: every £500k of pre-money at a £250k raise changes investor ownership by roughly 2–3 points; every 1 point given to the IP settlement is 1 point less founder control at exit.

2.3 Treatment of David Agunede / the MaternaLink IP — decision required

The relationship is undocumented (VERIFIED gap, FACTS_BASE §1). What is known: he signs as "Founder & CEO, MaternaLink"; he sent the Ekiti proposal (March 2026) and forwarded a prototype (September 2026); who wrote the prototype code and who owns the brand, decks and proposals is unknown. The decision below must be preceded by an **IP audit** (a one-page questionnaire and a code-repository review): authorship of code, third-party components and licences, any employer or university claims, any co-authors, whether any real patient data has been processed.

3. Three options for the MaternaLink IP

Dimension	Option 1 — IP assignment for equity	Option 2 — Licence to Vytalix	Option 3 — Joint venture
Mechanism	David Agunede assigns all MaternaLink IP (code, brand, decks, proposals, prototype, domain, data) to Vytalix Group Ltd in exchange for vested ordinary shares and, optionally, a defined role (e.g., Head of Product Africa / Country Lead Nigeria).	He retains ownership; grants Vytalix an exclusive, worldwide, perpetual (or long-term) licence with a right to sublicense, plus an option to purchase; consideration is a small equity stake and/or a royalty on MaternaLink revenue.	New company (e.g., MaternaLink Ltd) owned e.g. 60/40 or 51/49 by Vytalix and David Agunede; IP contributed by him, cash/services/brand contributed by Vytalix.

Dimension	Option 1 — IP assignment for equity	Option 2 — Licence to Vytalix	Option 3 — Joint venture
Investor view	Clean: the company owns what it sells. Standard for pre-seed.	Acceptable only with an exclusive, irrevocable licence and an option to acquire; investors will discount valuation and may require conversion to assignment as a condition of funding.	Negative: minority partner in the core asset, two cap tables, deadlock risk; most UK angels and SEIS funds will decline.
Speed / cost (ESTIMATE)	3–6 weeks; £2.5k–£6k legal (assignment deed, shareholders' agreement amendments, vesting).	3–6 weeks; £3k–£7k (licence drafting is more bespoke).	8–16 weeks; £8k–£20k (new entity, JV agreement, IP contribution agreement, funding waterfall).
Alignment	High if vesting and role are tied to product milestones.	Medium: he keeps a separate interest that can conflict (e.g., licensing to others after term, or disputes over scope).	Low–medium: separate governance, incentives can diverge (e.g., Nigeria-only ambitions vs UK-first strategy).
Downside protection for Vytalix	Warranties on authorship and originality; indemnity; reverse vesting if he leaves within cliff.	Termination rights on breach; but core asset can revert.	Deadlock and exit provisions; still exposed to a partner's unrelated liabilities.
Downside protection for David Agunede	Equity with vesting; a defined role; an anti-dilution floor is NOT recommended (investors will strip it).	Retains ownership; royalty income independent of equity.	Board seat; ownership retained via the JV.
SEIS/EIS impact	None if shares are ordinary and issued for the IP at a fair value (TO VALIDATE valuation treatment for shares issued for non-cash consideration).	None.	Complex: the JV may not be a qualifying subsidiary if Vytalix holds < 90% (TO VALIDATE).
Tax note (TO VALIDATE)	Shares issued for IP may create a taxable disposal for him; a section 431 election is usually advisable for restricted shares; obtain accountant advice.	Royalties are income for him; withholding considerations if he becomes non-UK resident.	Both parties need tax advice; possible stamp duty and VAT points on contributions.
Recommendation	Preferred.	Fallback if he will not assign: exclusive licence with an option to acquire for shares exercisable at any time within 36 months.	Not recommended. Use only if a funder insists on a Nigerian-owned vehicle for a specific contract — and then as a Nigerian delivery JV, not as the IP-holding entity.

3.1 Proposed heads of terms for Option 1 (to be negotiated; every number PROPOSED)

Term	Proposal	Rationale
Consideration	8–12% ordinary shares in Vytalix Group Ltd (pre-pool, pre-investment), subject to IP audit clean-up; top of range only if he takes an operational role	Reflects a concept, a proposal and a prototype of unverified maturity with no data or validation (FACTS_BASE \$1). A functioning, audited codebase and a live government relationship that can be evidenced would justify the top of the range.
Vesting	Reverse vesting 4 years, 12-month cliff; 25% vests immediately on signing if the IP audit is clean and the code is transferred to a Vytalix repository	Recognises contribution to date while protecting the company.

Term	Proposal	Rationale
Role	Defined title, hours, reporting line to the founder; if operational, a consultancy or employment agreement with IP and confidentiality clauses	Prevents "two CEOs" — the Ekiti signature block must change immediately after signing.
Warranties	Sole authorship or full chain of title; no third-party rights; no real patient data held or processed; no prior commitments to Ekiti or any third party that bind the IP	Standard; essential given the unsolicited government proposal.
Non-compete / non-solicit	12 months post-leaving, limited to maternal digital health in named territories	Enforceable only if reasonable — solicitor to advise.
Ekiti and other government leads	All existing and future government conversations conducted in Vytalix's name; his relationship contributions recognised in vesting milestones	Prevents parallel proposals in "Maternal Link" name.
Brand	"Maternal Link" / "MaternaLink" name and any domain/handles transferred; trademark filed by Vytalix	Name consistency; one spelling to be chosen — "MaternaLink" is used across this repository (ASSUMPTION).
Dispute resolution	Mediation then arbitration (London); governing law England & Wales	Cost containment.

3.2 Solicitor instruction — FLAG

A solicitor must be instructed before any term is offered. Scope: (1) founder IP assignment; (2) David Agunede IP assignment deed and shareholders' agreement; (3) articles amendment for vesting and leaver provisions; (4) template contractor IP clause; (5) name/trademark advice (§7). Budget ESTIMATE £4k–£9k for the package with a fixed-fee start-up practice; do not use a general high-street firm for the IP deed. Interim step available in week 1 at near-zero cost: a signed **mutual standstill and confidentiality letter** confirming that neither party will license, sell or present MaternaLink to third parties for 60 days while terms are agreed.

4. Governance

4.1 Statutory board (PROPOSED phasing)

Phase	Composition	Reserved matters	Cadence
Months 0–6	Founder (sole director) + one independent non-executive director (unpaid or £3k–£6k/yr ESTIMATE; ideally a former NHS or health-tech executive)	Issue of shares; borrowing > £25k; any contract > £50k; any clinical or regulatory claim in external material; any government proposal; hiring above £60k salary	Monthly 60-min board; written resolutions in between
At pre-seed	Founder; independent NED; investor director or observer (if round lead requires)	Add: annual budget; option grants; related-party transactions (Kemek Enterprise conflicts must be declared and minuted)	Monthly, with a quarterly strategy session
At subsidiary creation	TopCo board as above; subsidiary board = founder + product lead + clinical/regulatory NED	Product: intended-use statement changes; any feature that could cross into medical-device territory; data-sharing agreements	Monthly

Policies to adopt at incorporation (templates in [/operations](#), TO CREATE): conflicts of interest register (the founder's other venture, Kemek Enterprise Ltd, is a declared interest — KNOWN); anti-bribery (UK Bribery Act

applies to Nigeria dealings); data protection; clinical claims sign-off; delegated authorities.

4.2 Advisory board — six profiles to recruit (none appointed today: KNOWN)

Compensation: 0.25–0.5% options vesting over 24 months with monthly cliffs, or £500–£1,000 per meeting for those who cannot hold equity (NHS-employed clinicians often prefer a fee or nothing; check trust conflict-of-interest rules). Expected commitment: one 90-minute meeting per quarter plus ad-hoc calls. Advisors are not decision-makers and are never described as "partners" or as endorsing the product.

#	Profile	What they unlock	Where to find them (routes, not names)	Priority
1	Consultant obstetrician or maternal-medicine consultant with quality-improvement or MBRRACE/Ockenden-response experience	Clinical credibility; intended-use statement; content sign-off; pilot introductions	RCOG-affiliated QI leads; authors of trust maternity improvement plans; academic obstetrics departments	Month 1–2
2	Senior midwife: Head/Director of Midwifery, digital midwife or Maternity Digital Lead	The real buyer's perspective; workflow reality; DCB0160 on the trust side; RCM network	Digital midwife networks; Maternity and Neonatal Voices Partnership leads; LinkedIn "digital midwife" titles	Month 1–2
3	NHS digital / commissioning leader: former CNIO, ICB digital director, or Health Innovation Network commercial lead	Procurement reality (DTAC, G-Cloud, frameworks); ICB priorities; introductions to LMNS leads	Health Innovation Network alumni; ex-NHS England transformation staff now consulting	Month 2–4
4	Nigerian health-system leader: former state Commissioner for Health, SPHCDA executive, or a senior figure from a national maternal-health programme	Credible entry to states beyond Ekiti; donor relationships; realism on pricing and politics	Nigeria Health Watch networks; alumni of MSD for Mothers or GFF-funded programmes; diaspora clinical associations in the UK	Month 2–4
5	Regulatory and clinical-safety adviser: qualified Clinical Safety Officer (DCB0129/0160), with MHRA SaMD and UK GDPR/DPIA experience	Keeps v1 outside device scope; hazard log; DTAC; grant credibility	Digital health consultancies; independent CSOs; NHS informatics networks	Month 2–3
6	Health-tech operator/investor: founder or CEO who has scaled a UK digital-health company to Series A+, ideally with women's-health or emerging-market exposure	Fundraising, pricing, hiring; investor introductions	Angel syndicates focused on health; women's-health founder communities; accelerator mentor pools	Month 3–6
6b (optional)	Lived-experience and community voice: leader from a Black maternal-health advocacy organisation, engaged as a paid adviser not a token	Trust, language, co-design; protects against the "built about us without us" failure	Community organisations (no relationship claimed); approach with a paid brief	Month 3–6

Recruitment method: 30-minute discovery conversation → written advisory brief → 3-month trial → letter of appointment. Announce advisors only after written consent to be named.

4.3 Decision rights (RACI summary)

Decision	Founder/CEO	Board	Advisory board	Division P&L owner
Strategy, budget, fundraising	R/A	A (approve)	C	C
External clinical/regulatory claims	R	A	C (clinical adviser sign-off required)	I

Decision	Founder/CEO	Board	Advisory board	Division P&L owner
Pricing changes > 20%	R	I	C	A
New market entry	R	A	C	C
Hiring within budget	A	I	—	R
Government proposals	R	A	C	C

5. Division charters

Element	Vytalix Advisory	Vytalix Consulting	Vytalix Health Solutions	Vytalix E-Learning
Mission	Give health leaders and founders the strategic judgement to make technology work in real care settings	Deliver the analysis, business cases and readiness work that turn strategy into funded, procurable programmes	Build MaternaLink (in development): a non-diagnostic maternal care-coordination, communication, education and monitoring-support platform for NHS and low-connectivity settings	Turn Vytalix's practice into learning that health workers, founders and organisations can apply the same week
Offer	Founder-led advisory days and monthly retainers; fractional Chief Strategy/Digital Officer; board advisory; investor and grant readiness	Ten fixed-price sprints (01_BUSINESS_MODEL.md \$5): discovery, business case, digital maternity readiness, UK market entry, Africa market entry, DTAC/evidence readiness, health inequalities review, evaluation	MaternaLink v1 MVP (12–16 weeks from funded start), design-partner pilots, then site licences (UK) and per-woman programme licences (Africa); v3 AI research track gated by validation	Four self-paced courses, subscription library, corporate and university licences, co-branded NGO programmes
Customers	Health-tech founders (pre-seed–Series A), SME providers, NGOs, African ministries via donors, investors/accelerators	NHS trusts/ICBs (via frameworks or sub-contract), private providers, health-tech vendors, NGOs/foundations	NHS trusts and LMNS, private maternity, employers/insurers; African state governments, NGOs, HMOs	Midwives, nurses, health-tech professionals, students (UK and Africa); employers; universities
Primary KPIs	Billable days/month (target 8–10 by month 6); retainers active (3 by month 9); day-rate realised (≥ £1,200); NPS; referral rate	Sprints sold/quarter (2 by Q2); gross margin ≥ 55%; on-time delivery 100%; reusable assets per engagement ≥ 1; NHS references (1 by month 12)	Gates closed (IP, prototype audit, CSO appointed, DPIA); design partner signed (month 9); MVP usability score; hazard log maintained; first paid pilot (month 12–24)	Courses live (2 by month 9); learners; completion rate ≥ 40%; CAC < £60; corporate licences (2 in year 1)
P&L owner	Founder/CEO	Founder/CEO until a Consulting Director is hired from services surplus (month 9–15)	Product Lead / CTO (fractional from month 4–6; equity-based)	Founder until a Learning Lead is contracted (month 9+)
Revenue label	ESTIMATE (£55k–£210k combined services, year 1)	as Advisory	£0 year 1; grants £50k–£150k possible	£20k–£47k year 1 from month 9 (ESTIMATE)

Element	Vytalix Advisory	Vytalix Consulting	Vytalix Health Solutions	Vytalix E-Learning
Constraints	Conflict-of-interest register; no advice on MaternaLink competitors' procurement without disclosure	No patient data without DSPT; PI insurance	No clinical, diagnostic or predictive claim; CSO sign-off on scope	No accreditation claims until granted

Shared services: brand, website, CRM, finance, legal, data protection — cost-allocated to divisions by headcount/revenue share from month 6.

6. Vision and mission

6.1 Vision statement (PROPOSED)

Technology for Life. A world in which every person — beginning with every mother — is connected to care that knows them, hears them and acts in time, wherever they live.

6.2 Five mission variants

Each variant answers: what problem; whom we serve; why we exist; how technology helps; why investors care; why healthcare organisations care. Claims are constrained by FACTS_BASE §4 (no "proven", no "NHS-approved", no "reduces mortality").

(a) One sentence

Vytalix builds and advises on technology that keeps people connected to the care they need — starting with mothers in the UK and Africa.

(b) 25 words

Vytalix is a UK health-technology group that helps health organisations, founders and governments make technology work in real care — beginning with maternal care coordination.

(c) Website (about 80 words)

Care fails most often between people: between a woman and a changing set of professionals, between one shift and the next, between a community and a hospital. Vytalix exists to close those gaps. We advise and deliver for health organisations, founders and public bodies, and we are developing MaternaLink — a care-coordination and communication platform for maternity services built for both the NHS and low-connectivity settings. Technology for Life: practical, safe, and designed with the people who use it.

(d) Investor (about 110 words)

Maternal care is a coordination problem before it is a clinical one: inquiries in the UK repeatedly find failures to listen, escalate and hand over, and in Nigeria the largest maternal-death burden in the world is driven by delays in seeking, reaching and receiving care. Vytalix is a UK health-technology group with a cash layer — advisory, consulting and e-learning that fund the company from month two — and an equity layer: MaternaLink (in development), a non-diagnostic care-coordination platform designed for NHS trusts and for state and NGO programmes in Africa, with a research track towards validated risk insight. We are pre-seed and pre-formation; we sell the truth of our stage and a disciplined plan to earn the right to the next one.

(e) Corporate / healthcare-organisation (about 90 words)

Vytalix works with maternity services, providers, commissioners and programmes to make digital tools genuinely usable in the ward, the clinic and the community. We bring strategy, delivery, product development and learning under one roof, so that a business case, a readiness review, a pilot and a training programme are designed to fit together. MaternaLink (in development) is intended to support — not replace — clinical judgement: helping women stay connected to their care team and helping teams hand over safely. We are explicit about evidence, safety and what remains to be validated.

6.3 Values in one line each (PROPOSED)

Truth before traction · Safety before speed · Build with, not for · Both markets, one standard · Evidence is the product.

7. Name and trademark risk — and the clearance action

7.1 Risk assessment (based on FACTS_BASE §1, VERIFIED via search; registry status TO VALIDATE)

Conflicting mark / entity	Where	Sector	Nature of risk	Severity
Vytalix Inc. (vytalix.io)	Austin, Texas, US	AI/blockchain health-tech	One-letter difference, same sector; US expansion or US investors would confuse; possible USPTO objection if Vytalix ever files in the US	High (US), Medium (UK)
Vytalize Health	New Jersey, US	Value-based care	Phonetic similarity; same broad sector; investor confusion	Medium
VITALIX LTD (two UK companies, nos. 16813455 and 15155964 per FACTS_BASE)	UK	Unknown (TO VALIDATE SIC codes)	Companies House "same as / too like" rules could block "Vytalix"; if either holds a UK trade mark in Class 9/42/44 the risk becomes high	Medium–High
VITALX LTD; VYTAL(UK) LIMITED	UK	Unknown	Lower similarity; monitor	Low

7.2 Clearance action (owner: founder; complete by Day 10)

Step	Action	Tool / cost (ESTIMATE)	Pass criterion
1	Companies House name availability and "too like" check for "Vytalix Group Ltd" and reserves	Free, gov.uk company name availability checker	Name available and no "same as" objection
2	UK IPO trade-mark search (word and phonetic) in Classes 9 (software), 35 (business consultancy), 41 (education), 42 (SaaS), 44 (medical/health services)	Free search; filing later ~£170 for one class + £50 per additional class online (TO VALIDATE current fees)	No identical or confusingly similar live mark in Classes 9, 42, 44
3	EUIPO and USPTO (TESS successor) knock-out search for "Vytalix", "Vytalix", "Vitalix"	Free	Note conflicts; decide whether the US is a market within 5 years (it is not in the entry sequence — see 05_MARKET_STRATEGY .md)

Step	Action	Tool / cost (ESTIMATE)	Pass criterion
4	Domain and handle audit: vytalix.com/.co.uk/.health/.io; LinkedIn, X, Instagram	~£20–£200/yr per domain	.com or .health available or acquirable
5	Solicitor/trade-mark attorney opinion (30 minutes) on the four collisions above	£300–£600	Written "low risk" opinion, or a rename recommendation

Decision rule: if steps 1–2 pass and step 5 is "low risk", file UK trade mark for "VYTALIX" (word) in Classes 9, 41, 42, 44 immediately and incorporate; if any fails, incorporate under a reserve name within 5 days rather than delay incorporation. Reserve names (PROPOSED; all TO VALIDATE): "Vytalix Health Technologies Ltd" (keeps brand, adds distinctiveness), or a new coined mark selected from the brand system's naming criteria. The repository spelling "Vitalyx" is retired regardless (FACTS_BASE A1). The product mark "MaternaLink" needs the same five-step check (Class 9, 42, 44) — note the "Maternal Link" spelling used by David Agunede in the Ekiti email must be reconciled in the IP agreement.

Priorities / Risks / Next actions

Priorities

1. Sign the standstill letter with David Agunede this week; complete the IP audit questionnaire; instruct a solicitor on Option 1 heads of terms (fallback Option 2).
2. Clear the name (five steps) and incorporate Vytalix Group Ltd by Day 10; founder IP assignment on the same day.
3. Apply for SEIS/EIS advance assurance; open bank account; register with ICO; obtain PI insurance.
4. Recruit advisory-board profiles 1, 2 and 5 within 60 days (clinical credibility and safety are the gating items for every pilot conversation).
5. Adopt the division charters and reserved-matters list at the first board meeting.

Risks

- David Agunede declines to assign or licence → MaternaLink cannot be in the equity story; fallback is a clean-room rebuild of v1 under Vytalix with no reuse of his materials (feasible because v1 is re-scoped, but costs 3–6 months and the Ekiti relationship).
- Name conflict discovered after brand spend → mitigated by the Day 10 decision rule.
- SEIS/EIS advance assurance refused because the trade looks like consultancy (excluded activities include certain financial and legal services, not management consultancy; but HMRC scrutinises "trading" substance — TO VALIDATE) → structure the application around the product R&D and services trade together.
- Governance theatre: advisors named without consent or before appointment → reputational damage; rule: written consent, always.
- Founder concentration and the Kemek conflict → declared interest, time-allocation rule, board oversight.

Next actions

- Day 1–3: standstill letter; IP audit questionnaire sent; name searches steps 1–4.
- Day 3–5: solicitor engaged (fixed fee); trade-mark attorney opinion.
- Day 5–10: incorporation; founder IP assignment; bank; ICO; SEIS/EIS application drafted with accountant.

- Day 10–30: heads of terms with David Agunede signed; advisory-board outreach (10 approaches for profiles 1, 2, 5).
- Day 30–60: IP assignment deed executed; option pool approved; first board meeting minutes; charters adopted; this document re-labelled from PROPOSED to ADOPTED where executed.